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June 16, 2025

Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
2500 Security Boulevard
Baltimore, MD 21244

Dr. Thomas Keane
Assistant Secretary for Technology Policy (ASTP)
National Coordinator for Health IT (ONC)
Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

Re: Request for Information: Health Technology Ecosystem CMS-0042-NC (Docket CMS-2025-0050-0031)

Dear Administrator Oz and Assistant Secretary Keane:

Thank you for the opportunity to comment on CMS's efforts to modernize the digital health ecosystem and empower Medicare beneficiaries. We commend CMS's leadership in fostering innovation, strengthening interoperability, and creating a patient-centered approach to care. Below, we provide recommendations and comments on behalf of Johns Hopkins Medicine to address key areas outlined in the RFI, leveraging existing examples and identifying opportunities for impactful advancements in digital health and related technology.

Recommendations

1. Create More Flexibility for Cross-State Licensure

As with many technology opportunities, one of the most meaningful barriers is a non-technical one. To ensure access to healthcare for Medicare beneficiaries, the ability to deliver care across state lines is becoming increasingly important. This is especially crucial for patients in rural or underserved communities, areas with persistent provider shortages, and those with rare or complex conditions requiring specialized care. Unlocking this capability can help fulfill the promise of delivering care to the right patient at the right time, regardless of their location or geography.

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Telemedicine has proven to be an invaluable tool that improves access to healthcare. However, state-based licensure restrictions create significant barriers for providers seeking to deliver remote care across state lines. These regulations force physicians and other healthcare professionals to navigate a patchwork of state-specific rules, often requiring duplicative licensing processes that increase administrative burdens and costs without improving the quality or safety of patient care. As the nation transitions to more mobile and digital healthcare models, Medicare beneficiaries are at risk of losing critical telemedicine services if policy changes are not implemented soon. CMS has the unique opportunity to address these challenges by reducing regulatory barriers to cross-state practice, enabling providers to efficiently deliver care without compromising patient safety or system integrity.

CMS can spearhead cross-state care initiatives by collaborating with stakeholders to create a streamlined, uniform framework for licensure portability for cross state telemedicine. Specifically, CMS should explore partnerships with the Federal Trade Commission (FTC) and state medical boards to advance licensure reciprocity agreements for Medicare providers participating in telehealth services. Additionally, CMS could introduce federal standards for telemedicine licensure to eliminate duplicative requirements and harmonize regulatory expectations across states. This could include:

- Implementing federal guidance permitting cross-state telemedicine for continuity of care with established patient-provider relationships, valid for three years, matching the existing CMS definition of an established patient.
- Establishing funding to develop a national telemedicine provider registry to facilitate cross-state care for new patient relationships for rare conditions, complex care, and clinical trials.

This approach would dramatically reduce provider burdens while maintaining patient safety—ensuring that physicians meet robust quality standards at the national level. CMS could further incentivize state participation in such initiatives by tying licensure reform efforts to Medicare funding opportunities. By modernizing the licensure framework for telemedicine providers, CMS could not only improve access to care across state lines but also encourage innovation and competition in virtual care delivery models, ultimately lowering costs and improving outcomes for Medicare beneficiaries nationwide.

2. Certainty Around Reimbursement or Related Financial Models

Investments in digital health and technology-enabled care tools come with multi-year costs. The lack of long-term clarity around even basic telehealth coverage and associated reimbursement for Medicare beneficiaries restricts longer term investment in the broader digital health ecosystem, creates provider and patient confusion, and adds recurring administrative cost to prepare for impending terminations of coverage. Collaboration between Congress and CMS to ensure long-term clarity on telehealth reimbursement is an important first step to unlocking additional digital health care pathways into the future.

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3. Increase Access to Internet and Devices

Data from our own institution shows that patients enrolled in Medicare are much more likely to use audio-only versus video-based telehealth. Many Medicare beneficiaries across the country lack adequate internet access and devices, creating barriers to digital health adoption. Building on existing or previous models like the Affordable Connectivity Program, CMS could facilitate efforts to supply patients with free or subsidized internet service and digital devices such as smartphones or tablets. These initiatives could be expanded in collaboration with telecommunications companies or hardware vendors, ensuring healthcare access for underserved communities.

Currently, Medicare's current inducement rules limit the ability of healthcare systems and payors to directly support patients in accessing digital healthcare, creating unnecessary barriers to participation in telemedicine services and remote patient monitoring. Just as many Medicaid programs provide transportation support for patients needing to attend in-person visits, **Medicare should modernize safe harbor provisions to allow healthcare providers to supply patients with internet access and smart devices—such as smartphones, tablets, and computers—without fear of inducement penalties.** Importantly, providers should not be required to implement complex technical fixes that restrict devices solely to healthcare-related use, as such restrictions diminish the utility of the device for patients while placing an ongoing maintenance burden on health systems. By enabling healthcare systems to support patients in owning versatile devices that allow them to stay engaged with their healthcare team while also benefiting their daily lives, CMS can enhance digital access and empower patients to manage their care more effectively. Simplifying inducement rules would align federal policy with contemporary healthcare needs, ensuring that vulnerable populations have the tools to access telehealth, improve outcomes, and navigate the evolving healthcare landscape.

4. Fund Digital Literacy Training

Empowering Medicare beneficiaries requires not only providing access to technology but also ensuring they have the skills to use these tools effectively. CMS should invest in widespread digital literacy training programs targeted at seniors. Initiatives could include collaborative partnerships with community organizations, libraries, and health systems to offer training on how to navigate patient portals, use telehealth platforms, and manage digital health applications. Expanding digital literacy will not only improve user engagement but also enhance health outcomes by enabling beneficiaries to interact meaningfully with digital resources. This type of programing is essential to ensure that Medicare beneficiaries can use available healthcare technologies.

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5. Modernizing Language Translation Practices in Healthcare Through AI

Language translation and interpretation needs in healthcare have grown significantly, especially as more patients utilize digital tools like patient portals for communication. Current regulations, however, prohibit the use of AI-based technologies like ChatGPT for translation purposes, requiring all written healthcare communication to be reviewed by certified human translators. This approach not only increases costs but also compromises timely care and safety for patients who do not speak or read English.

Recent research published in *Pediatrics* suggests that AI-based translations, such as ChatGPT, may even outperform human interpreters in certain scenarios (Gurjal et al., 2024). This evidence highlights an opportunity to leverage AI to bridge communication gaps in healthcare while maintaining quality. Given the growing use of patient portals and messaging among Spanish-speaking populations, a task force should be convened to re-evaluate current regulations and provide common-sense guidelines. **Allowing AI-based tools for written translation in appropriate contexts, especially in low-risk scenarios, could enhance access to care, reduce costs, and improve safety for non-English-speaking patients.**

6. Streamline Consent Requirements for Digital Health and Patient Cost Share

CMS should simplify consent requirements for digital health services to reduce administrative burdens and accelerate digital tool adoption. Patients should have access to easy, transparent mechanisms to provide consent around data sharing and cost responsibilities. Though we appreciate specific areas of the digital health care ecosystem having structured reimbursement in the CMS fee schedule, the low dollar patient / beneficiary cost share creates a major operational barrier to what is increasingly shown as value-oriented care options. Eliminating these low dollar cost shares would permit faster uptake and decreased consent requirements. Streamlining current processes, similar to the approach applied to telehealth claims during the COVID-19 Public Health Emergency, could limit friction for patients while maintaining compliance with privacy and transparency standards.

7. Promote Research to Identify Valuable Digital Health Tools

CMS should invest in research partnerships with both research medical systems and organizations like the Peterson-KFF Health System Tracker to better understand the effectiveness, cost, and impact of digital health tools. For example, Peterson's diabetes tool report has provided valuable insights into which technologies genuinely improve outcomes. Funding more robust research will allow health systems to make data-driven investments in digital solutions that deliver real-world value for Medicare beneficiaries.

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8. Invest in AI for Actionable Remote Monitoring Data

Remote monitoring technologies are becoming pivotal in managing chronic conditions for Medicare beneficiaries, but the large quantities of data generated often overwhelm providers. The most common current alternative is to deploy (or contract with) a separate high cost, over-demanded clinical workforce – try to hire more providers – which is already in short supply. CMS could both invest in AI research and applications that analyze remote monitoring data to prioritize actionable insights for providers and work with appropriate agencies on a regulatory framework to ensure appropriate balance of liability between clinical team and AI tools. By harnessing AI, health systems can deliver more precise and meaningful care without increasing provider burden or compromising patient outcomes.

9. Foster Interoperability Through Additional Measures

For Medicare providers to fully embrace digital infrastructures, we agree with continued improvement in data interoperability standards. Reducing technical, financial, and bureaucratic barriers to EHR integration will allow patient data from wearable devices, third-party apps, and remote monitoring tools to flow seamlessly into clinical workflows. We applaud the continued work from CMS on frameworks for centralized interoperability standards to minimize fragmentation and inefficiencies.

We do believe there is meaningful distinction with safety, quality, and operational considerations for "read" ("data out") patient health data interoperability – which we continue to improve to ensure patients and Medicare beneficiaries have easier pathways to aggregate their own personal health data – and "write" ("data in") interoperability. Covered entities and their associated legal medical records represent that institution's care provided and associated data to support said care, and each entity and their providers need to retain control over when new data enters the record. The risks in removing each entity's control over write access to the record are immense, including creating an ambiguous, unverified legal medical record and an unknown clinical liability framework for "who knew what when" should new, clinically significant data enter the record without any responsible clinician's knowledge. Similarly, operational workflows tied to the medical record, such as scheduling, represent complex internal processes to try to balance patient needs, provider specialization, care team availability, financial clearance, and high quality, safe prioritization for urgent medical concerns. These processes are distinct from patient-centered access to their health data and we recommend continued focus on patient heath data, not internal health business processes and other administrative functions. We would oppose extending interoperability mandates to non-clinical data. We would look forward to continuing to work with CMS and the broader data interoperability ecosystem to ensure continued improvement in patient health data interoperability, and at the same time ensure safe process and governance over write-based access into legal medical records.

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In addition, CMS should build upon existing successes like the Interoperability and Patient Access Final Rule, exploring additional innovations for seamless data exchange while balancing innovation and liability by:

- Continuing to prioritize standards for APIs (application programming interfaces) to ensure compatibility across platforms and create incentives for API certification; support the ongoing expansion of the United States Core Data for Interoperability (USCDI) to promote more comprehensive and semantically interoperable data exchange
- Identifying ways to incentivize partnerships with third-party technology developers to address interoperability gaps and create user-centered solutions
- Providing formal guidance that indicates that covered entities and business associates are not responsible for breaches that occur when a regulated entity permissibly discloses information through a health information exchange or other interoperability mechanism based on the misrepresentation of another participant. In order to appropriately incentive participation in TEFCA and other health information exchange efforts, covered entities and business associates need greater assurances that abuses in the systems will not result in liability to them, as the disclosing entity. There has been an uptick in such abuses in national exchange networks, and in those situations, the regulated entities have been left with the burden of sorting through the chaos and the potential liability despite having done nothing wrong, while the bad actors and intermediaries/QHINs who failed to vet their participants adequately have faced no consequences. In order to truly achieve interoperability, incentives and liabilities need to align fairly, and the burden and repercussions need to be fairly allocated to those who violate the terms and conditions, not just to the regulated entities. Until the regulations align with this principle, there will remain hesitation to participate in TEFCA and other systems, and true interoperability is unlikely to be achieved.
- Regarding identity, there is meaningful difference between identity verification (which comes with numerous identity data points) versus identity / patient matching inside the various legal medical records – few of which hold the aggregation of identity data points for verification purposes. This creates a significant issue – the ability to verify identity but the risk of mismatching into legal medical records holding far fewer identity data points. Should third party identification strategies be prioritized, such as those mentioned in the RFI, we ask that there be an evolution of the liability framework should a third-party purport to match a verified identity into a medical record, and that match turn out to be incorrect. Today, liability is on the covered entity, which of course limits the use of third-party digital verification companies (as the problem remains patient match, not identity verification). In order to encourage adoption of digital identification and associated verification, we recommend an evolution of the liability

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framework to ensure the provider entity is held harmless if acting on a confirmed identity that turns out to be incorrect.

Conclusion

The modernization of Medicare's digital ecosystem represents a transformational opportunity to empower beneficiaries and improve system-wide efficiency. By addressing cross-state licensure, clarifying reimbursement and cost share, digital literacy, device access, AI adoption, and interoperability enhancements while streamlining administrative and research efforts, CMS can unlock the potential of health technology for millions of Americans. We appreciate CMS's leadership on these efforts and welcome collaboration to realize this vision together.

Thank you for the opportunity to contribute to this important initiative.

Sincerely,

Brian Hasselfeld, MD
Executive Medical Director, Digital Health and Innovation
Johns Hopkins Medicine